

1. Administrative & Study Identification

Full Study Title: Prospective Observational Multicenter Study of Patients With AH and CKD Markers in Kazakhstan (PROGRESS-CKD)
Short Title/Acronym: PROGRESS-CKD **Protocol Number:** D1843R00359
NCT Number: NCT06744335 **Sponsor Name:** AstraZeneca **Investigational Product:** N/A (Observational Study) **Phase of Development:** Phase N/A (Observational) **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To evaluate and describe the rate of Chronic Kidney Disease (CKD) diagnosis in patients with Arterial Hypertension (AH) and CKD markers in Kazakhstan. **Secondary Objectives:** To describe demographic and clinical characteristics for these patients, understand the proportion of patients with various risk factors for the development and progression of CKD, and evaluate routine therapy before and after the diagnosis of CKD.

2.2 Background & Rationale To address the gap in the timely diagnosis and management of CKD among high-risk patients with Arterial Hypertension. The study evaluates the real-world frequency of microalbuminuria, proteinuria, and reduced glomerular filtration rate (eGFR <60 ml/min/1.73 m²) to better understand the clinical trajectory, risk factors (such as age, gender, BMI, and cardiovascular risks), and routine treatment patterns in this population.

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Observational (Prospective and Retrospective Cohort) **Control Type:** Single-arm (N/A for observational) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration **Target N\$:** 1,282 patients
Number of Sites: Multicenter (Kazakhstan) **Estimated Study Start:** 20-Jan-2025 **Estimated Primary Completion:** 30-Apr-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age $\geq$ 18\$ years at the time of inclusion and patient signed and dated written informed consent form (ICF). 2. Established diagnosis of hypertension (SBP $\geq$

140\$ mmHg and/or DBP $\geq$ 90\$ mmHg when measured at two different visits). 3. Laboratory markers of CKD (eGFR <60 mL/min/1.73 m² and/or albuminuria/proteinuria: UACR $\geq$ 30\$ mg/g, uPCR $\geq$ 150\$ mg/g, etc.) measured during the period $\leq$ 12\$ months prior to inclusion and persisting $\geq$ 3\$ months between two measurements without a recorded diagnosis of CKD in the primary medical record prior to inclusion.

4.2 Exclusion Criteria 1. Absence of a signed ICF in patients in the prospective follow-up group. 2. Participation in any randomized controlled trial within 3 months prior to inclusion in this study or during participation in this study. 3. An established diagnosis of type 1 or type 2 diabetes mellitus, symptomatic chronic heart failure (CHF) NYHA functional class III-IV, or diagnosis of AH of secondary genesis.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: N/A (Observational study; based on routine clinical practice). **Route of Administration:** N/A **Duration of Treatment:** N/A (Routine therapy follow-up period).

5.2 Concomitant & Prohibited Therapy Permitted: Routine therapy before and after the diagnosis of CKD as prescribed by the treating physician. **Prohibited:** N/A

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -365 to 0 Retrospective review of ambulatory cards or medical records for CKD markers ($\geq$ 3\$ and $\leq$ 12\$ months prior to inclusion) Baseline Day 0 Informed Consent, Medical History, Baseline Demographic/Clinical Characteristics extraction Follow-up N/A Routine clinical practice visits and ongoing therapy documentation End of Study 2026 Final data extraction, Review of diagnosis updates and routine therapy

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Rate of CKD diagnosis in patients with Arterial Hypertension and CKD markers. **Measurement Method:** Retrospective and prospective analysis of primary medical records and ambulatory cards.

7.2 Secondary & Exploratory Endpoints - Demographic and clinical characteristics (age, gender, BMI, smoking habits, blood pressure, CV risks). - Proportion of patients with various risk

factors for CKD progression. - Routine therapy tracking before and after the diagnosis of CKD.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via routine clinical practice and medical records review as applicable for an observational study. **Serious Adverse Events (SAEs):** Handled according to routine pharmacovigilance reporting guidelines for observational real-world studies. **Data Monitoring Committee (DMC):** N/A (Non-interventional study).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Observational Cohort Set (all enrolled eligible patients). **Sample Size Justification:** Target N=1,282 is designed to provide statistically significant epidemiological estimates of CKD diagnosis rates and routine therapy patterns in the targeted AH population. **Handling of Missing Data:** Descriptive statistical methodologies; handling via standard epidemiological imputation or exclusion from specific variable analyses.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP), local legislation, and ethical guidelines for observational research. **Monitoring Plan:** Routine data review of primary medical records and source data verification. **Audit Trail:** Documentation of informed consent and rigorous data validation in eCRFs.

11. Study Identifiers & Governance

Primary ID: PROGRESS-CKD **Registry Number:** NCT06744335 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** PROGRESS-CKD Study **Official Regulatory Title:** Prospective Observational Multicenter Study of Patients With AH and CKD Markers in Kazakhstan (PROGRESS-CKD)

12. Clinical Framework (Adapted from Hyperkalemia to AH/CKD Specific)

Primary Condition: Arterial Hypertension (AH) and Chronic Kidney Disease (CKD) Markers **Diagnosis Threshold:** eGFR < 60 mL/min/1.73 m² and/or UACR ≥ 30 mg/g (persisting ≥ 3 months) **Medical Methodology:** Routine clinical practice screening **Optimization Target:** Timely CKD diagnosis rate and routine therapy initiation

13. Study Procedures & Methodology

Management Pathway: Real-world clinical pathway for AH and CKD screening **Education Protocol (HCP):** N/A (Observational study) **Education Protocol (Patient):** N/A (Observational study) **Quality Audit Mechanism:** Source data verification of retrospective and prospective medical records

14. Observation & Follow-Up Schedule

Total Duration: Follow-up through estimated study completion (approx. 15 months duration) **Onsite Visit Cadence:** Dependent on routine clinical care schedules (non-interventional) **Remote Monitoring Frequency:** Periodic eCRF data extraction **Checklist Review Interval:** End of study data extraction

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]					